

ENGLISH FOR WORK / SEPTEMBER 2026

# Medical Laboratory Technician English



## Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

### PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical laboratory technicians and clinical laboratory support staff.

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

# Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

## 01 Accessioning and unambiguous identifiers

Clarify similar laboratory identifiers without merging cases.

## 02 Specimen suitability and recollection language

Explain suitability wording without interpreting health status or teaching collection.

## 03 Quality-control status and reportable results

Explain why a completed run is not necessarily a released report.

## 04 Reagents, instruments, and service coordination

Coordinate equipment status without giving technical operating instructions.

## 05 Turnaround times and realistic updates

Explain laboratory timing without inventing a guaranteed deadline.

## 06 Preliminary, final, and amended reports

Repair an inaccurate version statement without interpreting results.

## 07 Critical notifications and acknowledged handoffs

Communicate a fictional result accurately and confirm its receipt by the stated recipient.

## 08 Referral laboratories and outstanding work

Give a send-out handoff with a clear remaining task and accepted owner.

## Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

## Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

# A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

## 0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

## 5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

## 12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

## 22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

## 35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

## 47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

## 55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

## Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

**LESSON 01 / FACILITATOR NOTES**

# Accessioning and unambiguous identifiers

Clarify similar laboratory identifiers without merging cases.

**CASE**

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

**Teach this move**

Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Vocabulary to check: accession number, requisition, specimen label, accessioning.

**Model response**

Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised wording prevents a reference guess from becoming an identity claim.

**Second-round challenge**

Say both cases came from the same clinic and ask whether that is enough to match them.

**Language-check answers**

1. A - accession number Correct. The accession number links the laboratory case and records.
2. A - transposed Correct. The last two digits have exchanged positions.

**05 • Conversations**

1. A314 or A341 - Fictional laboratory call using the accession case.

2. An order without a registered specimen - Fictional electronic order exists, but laboratory receipt has not been confirmed.

3. One request, an additional container - Fictional receiving enquiry about a second container; intended relationship is unresolved.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A shared surname on two requests

Two fictional requests have the same surname but different accession references. A caller uses the surname only. The technician can request the full required verification; neither case status has been disclosed.

Role A: Technician: explain why the surname does not identify one case.

Role B: Caller: you have the request reference but initially omit it.

Possible opening: I'm checking the Morgan sample from this morning.

### Success checks

- Ask for the required reference and identity verification.
- Avoid choosing a case by arrival order.
- Read back the established reference before discussing its status.

Add a complication: The caller assumes the earlier arrival must be the intended case.

## LESSON 02 / FACILITATOR NOTES

# Specimen suitability and recollection language

Explain suitability wording without interpreting health status or teaching collection.

**CASE**

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

**Teach this move**

Insufficient quantity concerns available sample, not a measured patient level. State the documented decision and its request reference. Do not generalize one specimen issue to every requested test.

Vocabulary to check: hemolysis, insufficient quantity, recollection, acceptance criteria.

**Model response**

For Q6, insufficient quantity describes the amount of specimen available, not a low result in the patient's blood. The laboratory lead has requested recollection through your clinic. There is no reportable result from this request in this case. Please confirm that the care team has received the recollection message; the collection arrangements belong with them.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. This prevents a suitability problem from sounding like a clinical finding.

**Second-round challenge**

Ask whether the collector definitely caused the problem; the cause is not established.

**Language-check answers**

1. D - specimen amount Correct. Quantity concerns how much specimen is available.
2. C - requested Correct. A request is not proof a new specimen was obtained.

## 05 · Conversations

1. Too little specimen, not a low result - Fictional Q6 call using the lesson case.
2. Hemolysis is not a diagnosis - Fictional enquiry about a specimen comment; method-specific assessment remains with the laboratory lead.
3. Recollection requested, arrival unknown - Fictional specimen-status call; clinic says a new collection was planned, not confirmed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | One rejected request, another still pending

A laboratory lead rejected specimen use for request J1 under documented acceptance criteria. Request J2 has no decision yet. The clinic assumes both were rejected. No technical cause or collection directions are supplied.

Role A: Technician: limit the rejection statement to J1.

Role B: Clinic nurse: ask whether J2 also needs recollection.

Possible opening: Should we tell the patient that both tests need another sample?

### Success checks

- State J1's documented rejection.
- Keep J2's decision pending.
- Ask the lead to clarify J2 without independently ordering recollection.

Add a complication: Both requests share a collection visit, but their suitability decisions differ in status.

## LESSON 03 / FACILITATOR NOTES

# Quality-control status and reportable results

Explain why a completed run is not necessarily a released report.

**CASE**

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

**Teach this move**

Describe the stage without teaching control interpretation or corrective testing. Unresolved review does not establish that every patient result is wrong. Keep the release decision with the laboratory's qualified reviewer.

Vocabulary to check: quality control, control material, analytical run, release authorization.

**Model response**

The analytical run has finished, but the quality-control review remains unresolved and release has not been authorized. I cannot describe these results as final just because the instrument completed its work. The laboratory lead is reviewing the issue. I can report that status to your team, but I do not have a confirmed release time.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. Instrument completion and reporting permission answer different questions.

**Second-round challenge**

Ask whether the quality review proves the patient results are inaccurate.

**Language-check answers**

1. C - unresolved Correct. Unresolved describes the outstanding review.
2. B - control material Correct. Control material monitors test-system performance.

## 05 · Conversations

1. Finished on the instrument, awaiting review - Fictional clinic enquiry using the quality-control case.
2. Control material is not a patient result - Fictional new colleague misreads the source of an entry; no numerical results.
3. A run includes more than one request - Fictional result-status query; one reviewed request does not establish the whole run's status.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | Review assigned but not completed

The laboratory lead has assigned quality review to reviewer Ana. Ana has acknowledged the task but has not reported a decision. A colleague wants to mark the run released.

Role A: Technician: distinguish review ownership from release permission.

Role B: Colleague: you believe Ana's acknowledgment finishes the review.

Possible opening: Ana accepted it, so can I mark the results released now?

### Success checks

- Specify that Ana accepted the review task.
- Keep the review decision pending.
- Avoid interpreting control values or authorizing release.

Add a complication: The colleague quotes the word accepted without naming what was accepted.

## LESSON 04 / FACILITATOR NOTES

# Reagents, instruments, and service coordination

Coordinate equipment status without giving technical operating instructions.

**CASE**

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

**Teach this move**

A service engineer's completed visit and the laboratory's permission to resume testing are separate. State the instrument reference, observed status, and exact question. Do not guess settings, restart methods, or cause.

Vocabulary to check: reagent, lot verification, maintenance record, downtime.

**Model response**

Could you confirm H2's return-to-service status? The service visit ended at eleven, but I do not see the laboratory lead's confirmation that testing can resume. I need to update the clinic accurately. Please distinguish the completed service visit from any checks or release decision still outstanding, and tell me who owns the next status update.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised wording does not turn a departure time into operational readiness.

**Second-round challenge**

Ask whether the instrument's illuminated screen proves it is available for testing.

**Language-check answers**

1. A - unconfirmed Correct. No laboratory confirmation has been supplied.
2. A - lot number Correct. The lot number identifies the material batch.

## 05 • Conversations

1. The engineer left at eleven - Fictional service-status call using the H2 case.
2. Received does not mean verified - Fictional reagent shipment; lot V8 received but lot verification is incomplete.
3. A service reference without patient details - Fictional support call about a display message; no patient data or repair steps are shared.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 • Say it: second scenario | A maintenance date from the wrong instrument

A colleague quotes a completed maintenance record for H1 while answering an enquiry about H2. H2 remains unavailable. The laboratory lead can review both references.

Role A: Technician: identify the scope mismatch and request H2's actual status.

Role B: Colleague: you saw a similar instrument name and assumed the record applied.

Possible opening: That maintenance entry names H1; the clinic is asking about H2.

### Success checks

- Repeat both instrument references clearly.
- Avoid transferring one instrument's completion status to another.
- Request the lead's H2 status without giving operating advice.

Add a complication: Both instruments are in the same room, which the colleague treats as confirmation.

## LESSON 05 / FACILITATOR NOTES

# Turnaround times and realistic updates

Explain laboratory timing without inventing a guaranteed deadline.

**CASE**

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

**Teach this move**

Turnaround intervals need defined start and end events. Correcting the starting point does not eliminate delays. STAT is a priority designation, not a promise that a particular result already exists.

Vocabulary to check: turnaround time, STAT priority, backlog, add-on request.

**Model response**

The quoted two-hour turnaround is measured from laboratory receipt, not collection. T5 was received at nine after collection at eight. That distinction changes the reference point, but the current backlog means I still cannot promise a report at eleven. The section lead can provide the latest estimate; please treat it as an estimate unless confirmed otherwise.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The timing explanation identifies both the clock and the uncertainty.

**Second-round challenge**

Ask whether adding STAT to your telephone message independently changes the recorded order priority.

**Language-check answers**

1. D - receipt Correct. Receipt is the stated starting event.
2. A - suitability review Correct. Presence alone doesn't establish suitability for extra testing.

**05 • Conversations**

1. Which clock starts at nine? - Fictional T5 timing call using the lesson case.

2. The add-on is a new request - Fictional clinic requests extra testing on an existing specimen; no technical suitability decision.

3. A priority label needs confirmation - Fictional clinician enquiry says urgent; recorded order priority has not been checked.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## **06 · Say it: second scenario | A revised estimate after a backlog update**

At 10:00 the section lead estimated reporting at noon. At 11:00 the lead says the estimate is no longer supported because the backlog changed. No replacement time is confirmed; a status update is agreed for 12:30.

Role A: Technician: correct the estimate without inventing a new deadline.

Role B: Clinic coordinator: you planned a noon results discussion.

Possible opening: We scheduled around noon. Has the report time changed?

### **Success checks**

- Identify noon as the superseded estimate.
- Keep the new report time unknown.
- State that 12:30 is a status-update commitment.

Add a complication: The coordinator treats 12:30 as the replacement report deadline.

## LESSON 06 / FACILITATOR NOTES

# Preliminary, final, and amended reports

Repair an inaccurate version statement without interpreting results.

**CASE**

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

**Teach this move**

A final label describes a reporting stage, not an assurance that a patient is healthy or that no correction could ever follow. Use the reference, issue time, and version together.

Vocabulary to check: preliminary report, final report, amended report, reference interval.

**Model response**

I need to correct my earlier description. The ten o'clock report was preliminary, not final. The final report was issued at two and supersedes that earlier version for this request. Please confirm that your team is viewing the two o'clock report. I can clarify the version and status, but clinical interpretation belongs with the responsible clinician.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. Reporting status and clinical interpretation must remain distinct.

**Second-round challenge**

Ask whether a value inside a reference interval settles the whole clinical question.

**Language-check answers**

1. A - Amended Correct. Amended identifies a revised issued report.
2. B - reporting stage Correct. Final identifies a reporting stage.

**05 • Conversations**

1. The two o'clock version - Fictional correction call using the lesson case.
2. A reference interval isn't the whole answer - Fictional patient-support call; no values or diagnoses are supplied.

3. An amended report needs a notified recipient - Fictional laboratory has issued an amended report; receiving clinic has not acknowledged the notification.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## **06 · Say it: second scenario | A portal upload mistaken for clinical review**

A final report is visible in the portal. The ordering clinician has not confirmed reviewing it or contacting the patient. A caller assumes visibility means the clinical discussion is complete.

Role A: Technician: explain report availability without implying clinical review.

Role B: Clinic coordinator: ask which follow-up task still needs a named owner.

Possible opening: It's in the portal, so can we close the patient-explanation task?

### **Success checks**

- Separate report availability, viewing, and explanation.
- Do not interpret the findings.
- Request confirmation of the clinical follow-up owner.

Add a complication: The caller treats a portal view count as evidence of understanding.

## LESSON 07 / FACILITATOR NOTES

# Critical notifications and acknowledged handoffs

Communicate a fictional result accurately and confirm its receipt by the stated recipient.

**CASE**

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

## Teach this move

The sender states the information, the receiver repeats it, and the sender confirms or corrects it. Analyte means the substance measured. Millimoles per liter (mmol/L) and milligrams per deciliter (mg/dL) are different units; do not interchange or convert them in these exercises. All critical designations are fictional lab-specific facts, not universal thresholds.

Vocabulary to check: critical result, read-back, notification log, escalation pathway.

## Model response

Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter. You repeated six point one; it is six point four. Please read back the complete result. That matches, thank you. The report was issued at two ten; I'll record your receipt at two twelve and contact extension 214.

## Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. A corrected number needs a fresh complete read-back, followed by the sender's confirmation.

## Second-round challenge

Ask whether six point four proves a diagnosis. The technician preserves the reported value and leaves clinical interpretation to the clinician.

## Language-check answers

1. B - The value is six point four Correct. This restores the stated value and requests confirmation.

2. B - That read-back matches the report Correct. The sender verifies the receiver's complete repeat.

## 05 • Conversations

1. Six point four, not six point one - All facts fictional: Lumen designates C7 potassium 6.4 mmol/L critical under its own criteria. Identity and Dr Shah's receiving authority are already verified. Report issued 14:10, call at 14:12, contact extension 214. No universal threshold, diagnosis, or treatment is implied.

2. The escalation contact accepts S9 - Fictional Lumen case S9: sodium 121 mmol/L is designated critical by this lab. Report issued 11:06. First contact was unanswered at 11:07; immediate local escalation reached verified receiving nurse Elena at 11:08, extension 305. These are case facts, not a required waiting interval. No universal threshold or treatment is implied.

3. The unit needs correcting too - Fictional Lumen case G4: glucose 48 mg/dL, meaning milligrams per deciliter, designated critical under this lab's criteria. Identity and receiving nurse Morgan's authority are verified. Report issued 13:40; Morgan's extension is 306. No universal threshold, diagnosis, or treatment is supplied.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 • Say it: second scenario | A decimal lost in the repeat

Fictional Lumen case M2 has magnesium 0.8 mg/dL, meaning milligrams per deciliter, designated critical by this lab. Case identity and Dr Rao's receiving authority are verified. Dr Rao initially repeats eight, omitting zero point. No diagnosis, treatment, or universal threshold is supplied.

Role A: Technician: state M2, magnesium, zero point eight milligrams per deciliter; correct the repeat and verify the final read-back.

Role B: Dr Rao: initially say eight, then repeat the complete corrected result and accept receipt.

Possible opening: For verified case M2, Lumen's critical magnesium result is zero point eight milligrams per deciliter.

### Success checks

- Correct eight to zero point eight without converting units.
- Obtain M2, magnesium, zero point eight milligrams per deciliter in one read-back.
- Confirm the accurate repeat and recipient acknowledgment without interpreting the result.

Add a complication: Dr Rao asks whether eight and zero point eight are merely different ways of saying the same value; they are not.

## LESSON 08 / FACILITATOR NOTES

# Referral laboratories and outstanding work

Give a send-out handoff with a clear remaining task and accepted owner.

**CASE**

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

**Teach this move**

A manifest lists intended contents; parcel delivery does not itself confirm item acceptance. Chain-of-custody requirements apply only in their proper context, not automatically to every routine specimen.

Vocabulary to check: referral laboratory, send-out test, shipping manifest, chain of custody.

**Model response**

Eli, X8 is listed on Monday's manifest, and parcel delivery was confirmed Tuesday. We still lack the referral laboratory's accession acceptance for X8, so a result date is not established. Please take the acceptance enquiry and the clinic update agreed for three. Confirm that you have both tasks; parcel delivery alone does not close this send-out follow-up.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. This identifies the item-level gap and the communication still owed.

**Second-round challenge**

Ask whether a courier signature counts as referral-laboratory accession acceptance.

**Language-check answers**

1. A - intended contents Correct. A manifest identifies listed shipment contents.
2. B - both Correct. Both explicitly covers the two named tasks.

**05 • Conversations**

1. A delivered parcel with acceptance still open - Fictional X8 shift handoff using the lesson case.

2. A manifest entry without receipt confirmation - Fictional send-out coordinator compares a shipment list with the external receiving log.

3. Custody wording has a specific purpose - Fictional laboratory staff clarify documentation terminology without teaching collection or forensic procedures.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## **06 · Say it: second scenario | The receiving laboratory changes its contact person**

Referral laboratory contact Ana has moved roles. Send-out Z3 remains pending. Eli has not accepted the query. The receiving laboratory's service coordinator can assign a new owner; a clinic update is due at 16:00.

Role A: Technician: request a named new contact without claiming the test stopped.

Role B: Service coordinator: you can accept the ownership enquiry, but cannot confirm result timing.

Possible opening: Ana no longer handles Z3. Who has accepted the follow-up now?

### **Success checks**

- Separate contact ownership from specimen suitability.
- Keep result timing unknown.
- Preserve the four o'clock update while resolving who owns the external query.

Add a complication: The clinic assumes a staff change means its specimen must be recollected.

# Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

## Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

## Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

## Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

## Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

## Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

## Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

## Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

# Use the language responsibly

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## Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

## Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

### Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

### Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

### U.S. Bureau of Labor Statistics: Clinical Laboratory Technologists and Technicians

<https://www.bls.gov/ooh/healthcare/clinical-laboratory-technologists-and-technicians.htm>

### Centers for Disease Control and Prevention: About Laboratory Quality

<https://www.cdc.gov/lab-quality/php/about/>

### Agency for Healthcare Research and Quality: TeamSTEPPS Check-Back (or Repeat-Back)

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/checkback.html>

## Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.